

Boarding mentors practical handbook

Inspired South Africa

Course companion 2026

For school and Group approval before operational use

Using this handbook

Use this concise reference to prepare for duty, respond to concerns and hand over safely. It accompanies Boarding Mentor Essentials and must be read with the full standard, adopted school and Group policies, individual care plans and local procedures.

Status and authority

The source is Inspired South Africa Boarding Mentors Standards, Staff Handbook and Code of Practice, Version 1.1, July 2026. It requires legal, HR, safeguarding, nursing and health-and-safety specialist review before formal adoption. This companion does not confirm that approval has occurred and is not legal or clinical advice.

Must reflects a mandatory source requirement; should indicates expected best practice; may permits discretion exercised lawfully, reasonably and in boarders' best interests. Local procedures must be completed and reviewed at least annually. Where requirements differ, the higher safeguarding standard applies unless it conflicts with law.

Course completion alone does not authorise sole or unsupervised duty. Local induction, role-specific shadowing and formal competence assessment remain required. The source refers to Annexures C, E and L, which are not included in the supplied document; see page 12.

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If safety is uncertain, escalate. Protect immediate safety, summon the right help, stay within your role and record what happened.

Source front matter and definitions; sections 3, 7, 26 and 28

Prepare for duty

Confirm that you can work safely

- Required vetting must be complete, recorded and approved. Induction and formal competence assessment must be complete before sole or unsupervised duty.
- Maintain every qualification, licence, registration, clearance or practising status required for your role. Disclose a lapse, restriction, suspension or investigation immediately through the approved route.
- Report immediately if illness, fatigue, distress, alcohol, substances or medication may impair safe work. Do not begin or continue duty when unfit. The responsible manager must arrange safe relief without delay.
- Know whether you are on active duty, waking night duty, approved sleep-in duty, on call or off duty. Living on site does not mean continuous duty. Remain contactable throughout your assigned duty or on-call period.

Check deployment with the duty lead

- During active waking periods, no adult may be the sole responsible adult for more than 30 boarders. Above 30, at least two duty adults must be assigned and visible. Lower limits apply where age, risk or layout requires.
- Overnight, one designated duty adult must be physically present in each occupied boarding building, with a second trained adult on the same campus able to respond immediately.
- Two-adult emergency response capacity and a physically present trained first-aid responder are required whenever boarders are in residence. Confirm the formal 24-hour nurse-on-call route.
- Separate or distant areas need suitable coverage. Staff accompanying transport, trips or hospital visits are not also available for house supervision unless replacement coverage is explicitly provided. Only inducted, vetted and authorised people count as coverage.

These are Group safety controls, not universal legal ratios. Risk may require enhanced staffing or waking night duty; a numerical ratio never makes an unsafe arrangement acceptable.

Start of shift checklist

- ☐ Receive a structured handover and resolve missing information.
- ☐ Confirm expected and present boarders, leave, delayed returns and movements.
- ☐ Read relevant safeguarding, welfare, behaviour, dietary and medical alerts.
- ☐ Confirm staff locations, responsibilities, relief and emergency contacts.
- ☐ Check communication, access-control, first-aid and reporting systems.

Remain in your assigned area until properly relieved. Raise gaps in safe coverage immediately; do not silently absorb them.

Source sections 5 to 7, 12.1, 15.1 and 22.1

Professional boundaries

Be warm, reliable and fair. Every interaction should be open, accountable, proportionate and capable of being explained later. Boarders should have a network of support rather than an exclusive relationship with one adult.

One to one support and privacy

- Use a visible or auditable setting unless privacy is necessary for welfare, dignity or medical reasons.
- For a necessary private conversation, staff should use a room with visibility, keep the door open where appropriate, tell another adult and record a significant interaction.
- For late-night welfare support, call a second adult where risk or extended support is involved. Preserve accountability while protecting the boarder's dignity.
- Follow the approved room-entry and privacy procedure for bedrooms, bathrooms and changing areas, except in an emergency. Never make checks humiliating or unnecessarily intrusive.
- Staff homes are not ordinary meeting venues. Any authorised use of a shared public area must be specifically risk assessed and governed by local procedure.

Physical contact

Contact must be necessary, proportionate, age appropriate, sensitive to the boarder's wishes, professionally justifiable and transparent. Brief reassurance, first aid or practical support may be appropriate. Prolonged, intimate, unnecessary, hidden or staff-needs-driven contact is prohibited. Physical intervention has additional limits on page 5.

Digital conduct

- Use approved school platforms for duty and welfare communication. Do not privately message, follow or friend current boarders through personal social media.
- Never use secretive, flirtatious, emotionally intimate or overly personal messages. Do not send unrelated personal photographs, memes or jokes, conceal messages by deleting them, or post about boarders or internal matters without authorisation.
- If a boarder contacts a private account, preserve the message, avoid an extended conversation, redirect to the approved platform or emergency route and inform the appropriate leader. Act on any welfare or safeguarding concern.

Boundaries that must hold

- No favouritism, unexplained privileges, unusual access, secrecy, romantic or sexualised relationships, or emotional dependency.
- Do not share intimate details of your private life or use ridicule, humiliation, threats or emotional manipulation.
- Resident family members and guests must follow approved access rules and may not provide informal supervision.
- Record and report concerns about adult conduct through the approved safeguarding or HR route. Do not warn or investigate the adult informally, even if they are a friend or senior colleague.

Source sections 9.4, 10, 16.4 and 23 to 24

Routines and whereabouts

Know who is where

At all times, duty staff must know who is present or absent, where each boarder is expected, which adult is responsible and who has an elevated welfare risk. Never assume a boarder's whereabouts.

- Keep the approved sign-in and sign-out system current and auditable, with a backup during outages. Complete roll checks at defined points, including return from school, meals where required, curfew and lights-out.
- Follow up unexpected absence, delayed return or uncertainty immediately. Notify the senior boarding lead and activate the missing-boarder procedure without an extended informal search. Use page 7.
- Verify authorisation for exeat, weekend leave, appointments and changed collection arrangements. Never release a boarder to an unverified person or unsafe transport.

Make daily routines dependable

- Supervise actively. Avoid personal-device use or other distractions that reduce attention. Keep expectations calm, predictable, consistent and age appropriate.
- During study and prep, maintain a calm environment, support organisation, challenge persistent disengagement and report academic concerns. Do not replace subject teachers or offer unauthorised tutoring that creates dependency or favouritism.
- At meals and communal times, observe attendance, appetite, dietary needs, mood, inclusion and peer dynamics. Report food-quality, hygiene and special-diet concerns. Staff should eat with boarders where the school's supervision model provides for it.
- Follow published procedures for room checks, hygiene, devices, medication and lights-out. Night staff must be immediately reachable, complete required checks and record significant events before handover. Sleep is permitted only under an approved sleep-in arrangement.

Control access and movement

- Visitors must sign in, display identification where required and stay in authorised areas. Boarder visitors need prior approval; access to sleeping, bathroom or changing areas needs specific authorisation.
- Contractors must be supervised or restricted according to risk. Report lost keys, access failures, hazards and unsafe premises through the approved route.
- Only authorised, appropriately licensed and fit drivers may transport boarders in suitable, insured and roadworthy vehicles. Journey plans must cover supervision, seatbelts, contacts, medication, delays and destination handover.
- Private staff transport should be exceptional, authorised and recorded. One-to-one journeys need additional transparency and communication controls. Maintain house coverage when staff leave.

Source sections 5.2, 11, 12, 16 and 22

Wellbeing and fair behaviour

Notice changes and coordinate support

Look for changes in mood, sleep, appetite, hygiene, engagement, peer relationships and presentation. Notice withdrawal, coercion, exclusion, dependence on one adult, repeated rule-breaking or anxiety linked to particular people, places, weekends or online contact. Record patterns and share concerns promptly.

For mild homesickness, acknowledge feelings calmly, maintain routine and participation, support peer connection and practical reassurance, and follow the agreed parent-communication plan. Monitor whether distress improves. Escalate persistent or worsening distress; significant distress requires immediate escalation. Use safeguarding or medical routes as needed. Boarding staff are not therapists unless professionally appointed in that role.

Protect dignity and inclusion

Boarders have rights to safety, dignity, reasonable privacy, sleep, food, water, hygiene, healthcare, education, recreation and religious or cultural observance. They must know how to contact family, safeguarding, nursing, counselling and an independent adult, and how to complain without retaliation. Consider reasonable accommodation and challenge discrimination, bullying, initiation and exclusion.

Use fair consequences within authority

Consequences must be lawful, proportionate, fair, age appropriate and linked to behaviour. Stay calm, avoid public arguments and record serious incidents or patterns. Do not mislabel safeguarding or medical needs as misconduct. Placement, suspension and termination decisions belong to authorised leaders using the applicable process and safe care arrangements.

Prohibited sanctions include corporal punishment or threats; hazing or compulsory rituals; humiliation or public shaming; forced exercise, painful positions or degrading labour; collective punishment without established individual responsibility; punitive unsupervised isolation or locking a boarder in a room; and threats outside your authority.

Never deprive a boarder of food, water, sleep, medication, treatment, toilets, hygiene or safeguarding support. Confiscation must not prevent reasonable emergency or family contact without an approved alternative.

Searches and physical intervention

Searches must be authorised by policy and law, necessary, proportionate, dignified, appropriately witnessed and recorded. Intimate searches are prohibited. Obtain senior or police assistance where items, circumstances or resistance create risk.

Physical intervention is permitted only to prevent immediate harm and must be reasonable, proportionate and the least restrictive option. Never use it for punishment or routine compliance. Every use must be medically checked where appropriate, recorded, reported and reviewed.

Source sections 8, 13, 14 and Annexure K

Safeguarding and escalation

Report a safeguarding concern immediately. Proof is not required, and you must not assume someone else has reported it. If there is doubt, escalate.

Respond to a disclosure

1. Protect immediate safety and summon senior, safeguarding, medical or emergency support as needed. For child-on-child sexual harm, protect immediate safety and do not force mediation or treat a room change alone as resolution.
2. Listen calmly without leading questions. Do not promise secrecy, minimise, investigate beyond your role or confront the alleged perpetrator unless directed by the appropriate authority.
3. Explain that the concern must be shared with people who can help. Preserve relevant information and record the boarder's words, observed facts, time, actions and people informed.
4. Use the approved reporting route and confirm follow-up responsibilities. If the concern involves a senior leader or the safeguarding lead, use the alternative route without notifying the subject in advance.

Choose the earliest necessary response

Timing	Examples and response
Immediately	Any safeguarding concern or disclosure; abuse allegation; missing boarder; self-harm or suicidal concern; serious injury or acute illness; immediate risk or significant distress; serious adult boundary concern; serious overnight incident; fire, intruder or major security event.
Same day	A non-urgent behaviour or medical issue, repeated emotional distress, complaint or emerging pattern only when no safeguarding concern or immediate risk is present. A bullying allegation may require immediate safeguarding escalation; do not wait where harm is suspected.
Monitor and report	Mild homesickness, minor routine breaches or low-level conflict without risk. Record vulnerability, recurrence and patterns. Escalate if concerns persist, worsen or suggest harm.

The same-day and monitor categories never justify delaying safeguarding, uncertainty about whereabouts, significant distress or an allegation of misconduct. Every adult-conduct concern must be recorded and reviewed through the approved safeguarding or HR route.

Use the right authority

The Head of Boarding leads operations; the Designated Safeguarding Lead leads safeguarding decisions and records the external-reporting decision and basis; the medical or nursing lead gives clinical direction. The Executive Head retains school accountability. Use the local backup route if a lead is unavailable or conflicted.

Internal reporting does not replace legal reporting duties. Follow the approved procedure for relevant external authorities, Form 22 where applicable, SAPS, professional regulators and Group safeguarding. Do not assume reporting internally discharges every individual duty. Serious incidents also require the approved Group reporting route.

Source sections 3, 9, 19.2, 26 and 27

Missing boarder action card

Treat uncertainty about whereabouts as serious. Do not wait for the boarder to return or continue an informal search without escalation.

1. Confirm the absence and note the exact time and place the boarder was last seen.
2. Check the approved sign-out record, room, expected activity and known locations without delaying escalation.
3. Notify the senior boarding lead immediately and activate the local missing-boarder procedure.
4. Attempt contact through approved means. Gather concise, factual information from peers and preserve relevant CCTV or access information.
5. With the responsible lead, consider age, vulnerability, weather, medication, self-harm, substance, exploitation and transport risk. Maintain supervision for other boarders.
6. Follow the local procedure's thresholds for informing parents, security, SAPS and Group leaders. Use the named decision-maker and backup route.
7. Maintain a timed chronology of searches, contacts, decisions, people informed and changing risk until the boarder is found and safely assessed.
8. On return, arrange welfare, medical and safeguarding assessment before treating the event only as misconduct. Hand over follow-up and review actions to named staff.

Information to give the responding lead

- ☐ Boarder's identity, age and relevant known vulnerability.
- ☐ Last confirmed sighting, expected location and time absence was identified.
- ☐ Authorised leave or collection details and any inconsistency.
- ☐ Checks and contact attempts already made, with exact times.
- ☐ Known medical or safety-plan information needed for safe response.
- ☐ Current supervision arrangements and resources already called.

Local procedure must identify

Who leads the response, who coordinates searches and maintains other supervision, who contacts parents and police, the applicable reporting thresholds, and how to record return and welfare assessment. Keep the procedure and verified contacts accessible during power or network failure.

Procedure location _____

Senior lead and backup _____

Source sections 11.1 and 25.5; Annexure H

Self harm and acute distress action card

Do not leave a high-risk boarder alone and do not promise secrecy. Respond calmly and summon support. A mentor's role is immediate safety and prompt escalation, not clinical diagnosis or a formal clinical suicide-risk assessment.

1. Stay with the boarder where immediate risk is present. Listen and notice immediate danger without minimising their words or making promises you cannot keep.
2. Call the senior duty person, Designated Safeguarding Lead and nurse or medical support immediately. Arrange additional adult support and safe coverage elsewhere.
3. Remove immediate means of harm only where safe. Do not carry out an unauthorised search.
4. Use emergency services when risk is acute, clinical advice requires it, or safe care cannot be maintained on site. Do not let an unanswered on-call contact delay emergency response.
5. Follow the individual safety plan if one is available, within your role and training. Keep the boarder supported while appropriate help is arranged.
6. Record the boarder's words, observed facts, actions, times and people informed. Use the approved safeguarding and medical systems.
7. Ensure named staff take responsibility for clinical and pastoral follow-up, authorised parent communication, and review of the boarding safety plan. Provide a secure handover.

When distress is severe or worsening

Rapid deterioration, panic, trauma responses, eating concerns, recent injury or statements suggesting self-harm require prompt escalation. Do not treat significant distress as attention-seeking or a behaviour issue. Where there is immediate risk, use the immediate response above.

Preserve dignity and professional boundaries

- Use a calm, accountable setting and obtain a second adult where risk or extended support is involved.
- Share information with those who need it for safety and care. Do not promise absolute confidentiality.
- Do not become the boarder's only source of support. Coordinate with safeguarding, clinical and pastoral colleagues.

Safety plan location _____

Local emergency and clinical route _____

Source sections 13.3, 25.3 and 25.6; Annexure I

Health and emergency response

Know the clinical cover

Whenever boarders are resident, a trained first-aid responder must be physically present and a formal 24-hour on-call arrangement with a currently SANC-registered nurse must be in place. Know the primary and backup nurse, expected response, attendance criteria and emergency-service route. On-call nursing does not replace emergency services.

Read relevant individual healthcare plans and know how trained staff access emergency medication. An unwell boarder must not be left alone where clinical or safeguarding observation is required. Follow the sick-bay, overnight care, relief and parent-communication procedures; emergency care takes priority.

Medication duties have strict limits

- Only trained and authorised staff may administer medication, within competence and the approved system. Record administration immediately.
- Check authorised instructions, consent, identity of the medicine, expiry, secure storage and relevant records under the local procedure. Controlled medicines require enhanced security, count checks and an auditable register.
- Escalate omissions, refusals, errors, adverse reactions and discrepancies. Self-medication needs individual risk assessment and written authorisation. Never share, borrow, give medication informally or use it as a sanction.
- For a discrepancy, pause the discrepant administration or supply, secure it under the approved procedure, and get immediate nursing advice on the boarder's safety and next dose. Do not make independent treatment changes or delay emergency care. Do not alter or recreate records; inform the nurse and senior boarding lead immediately.

Seek immediate medical escalation

Triggers include breathing difficulty, severe allergic reaction, seizure, collapse or loss of consciousness; significant head injury or concussion red flags; suspected overdose or poisoning; severe pain, major bleeding, serious assault or possible fracture; self-harm injury, suicidal crisis or acute psychiatric deterioration; eating-related collapse, dehydration or significant refusal of food or fluids; or uncertainty that safe care can be maintained on site.

Respond to fire or a serious incident

- Protect life and immediate safety. Activate the relevant local emergency procedure, summon emergency services and senior support promptly, and act within your training.
- Follow designated roles for evacuation, lockdown, headcounts, vulnerable boarders and continued supervision. Use backup registers, medical alerts and contacts during outages.
- Preserve evidence, keep a timed chronology and record decisions and handovers. Do not conduct an unauthorised investigation.
- Use the nominated authorised communicator for parents and external stakeholders. Arrange post-incident support, review and corrective action without contaminating witness accounts.

Source sections 15 to 17 and 25.10; Annexure J

Records and parent communication

Record facts that another adult can act on

Records must be factual, dated, timed, attributable, objective and clear. Distinguish what you observed from what someone reported. Record actions, the people informed, their decisions and the follow-up still required. Avoid speculation and derogatory language.

- ☐ Attendance, leave, movement irregularities, missing incidents and delays.
- ☐ Symptoms, injuries, medication administration, omissions, errors and refusals.
- ☐ Behaviour, consequences, searches, confiscation and physical intervention.
- ☐ Bullying, safeguarding concerns, disclosures and allegations.
- ☐ Overnight disturbances, significant conversations, homesickness and emotional deterioration.
- ☐ Parent complaints, material communication and agreed actions.
- ☐ Any concern that may be part of a pattern or later become relevant.

Keep information in the right system

- Use approved systems with role-based access. Safeguarding, medical, disciplinary, employment and routine boarding records have different access needs.
- Do not keep sensitive information in personal notebooks, private cloud storage or unauthorised messaging apps. Share handover information securely and only with people who need it for care, safety or lawful management.
- Follow Group POPIA procedures for retention, correction, disposal and breach reporting. Do not invent retention periods or circulate another boarder's information.

Communicate within your authority

Be accurate, calm, timely, proportionate and appropriately confidential. Align sensitive details with the authorised leader before communicating, except where emergency information must be shared immediately. Express empathy or concern without speculation, prejudging an investigation or making admissions beyond your authority.

For a complaint, acknowledge and record the concern, refer it to the authorised leader and avoid promising an outcome. A complaint alleging harm, neglect, adult misconduct or safeguarding failure must go immediately through the safeguarding route, whatever the complaint is called. Never manage a serious complaint privately or discourage escalation.

Minimum serious incident record

Include the incident title, date, location, lead manager, safeguarding lead and boarders involved. Keep a timed chronology of observed or reported facts, actions, people informed and decisions. State current safety, next steps, record author and reviewer, external reports made and when Group was notified. Store the chronology securely with the official record.

Source sections 17.3, 18, 19 and 25.9; Annexure G

Minimum handover record

Use the approved handover system. This layout shows the minimum fields; do not use a personal copy to retain sensitive information. Refer to restricted records securely rather than duplicating unnecessary details.

Date and shift _____

Outgoing staff _____

Incoming staff _____

Boarders expected and present _____

On leave or delayed _____

Information and action required

Area	Information and action required
Safeguarding and welfare	
Medical and medication	
Behaviour and bullying	
Attendance and movement	
Parent communication	
Activities and transport	
Premises and security	
Outstanding actions and owner	

Handover completed at _____

Incoming staff confirmation _____

Before leaving, confirm the incoming team understands current risks, required checks, who has already been informed, and each outstanding action's owner and timing. Escalate unresolved safety concerns rather than leaving them only in a log.

Adapted from source Annexure F; sections 12.1, 18 and 24

Local contacts and readiness

Complete and verify this page through the school's authorised process before operational use. Keep an outage-accessible backup and review at least annually and after relevant changes.

Role or route	Name and contact or location
Senior boarding lead and backup	
Safeguarding lead and alternative route	
Primary nurse and backup nurse	
First aider on duty	
Emergency services and security	
Executive Head and Group incident route	
Counsellor and independent adult	
HR or speak up route and staff support	

Before independent duty

- ☐ Approved roster, relief plan, reporting lines and local contacts are known.
- ☐ Attendance, leave, collection, visitor, transport and privacy procedures are accessible.
- ☐ Missing-boarder, self-harm, medical, medication, fire, lockdown and other emergency procedures are understood and practised as required.
- ☐ Approved records, data access, parent communication and complaints routes are known.
- ☐ Required checks, role-specific shadowing, competence assessment and formal sign-off are complete.

Complete the source implementation pack

Annexure C (nurse-on-call service standard), E (formal competence sign-off) and L (annual self-audit) are referenced but absent from the supplied source. Leaders must obtain or approve the required local documents through the appropriate process. These checklists do not replace those tools or specialist approval.

Confirm the legal, HR, safeguarding, nursing and health-and-safety review required before formal adoption. Record the approved version and local authority below.

School and approved version _____

Authorised reviewer and date _____

Local procedures location _____

Source section 28; Annexure C, E and L references in sections 15.1, 7.1 and 20.1